

What the appendix contains

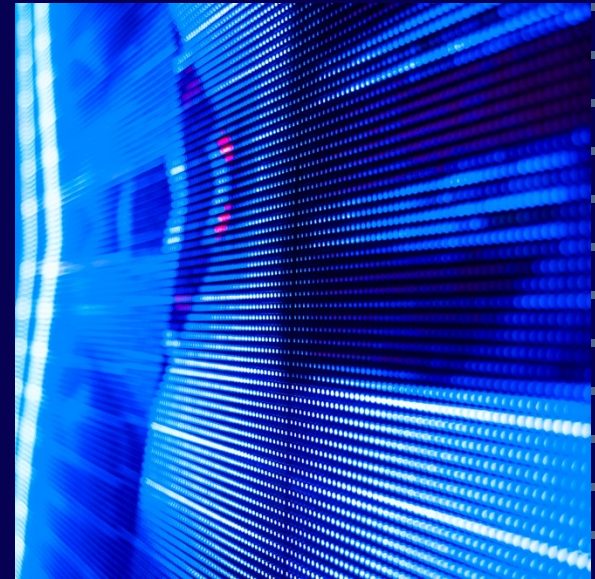
A1	Detailed close calendar	Day-by-day view of the current close for 14 entities	p. 36
A2	Entity readiness assessment	Scores for data, process and system readiness	p. 37
A3	Control framework	Key controls mapped to the target operating model	p. 39
A4	Tooling options	Comparison of three automation platforms	p. 41
A5	Assumptions and fee detail	Rate card basis, phase fees and exclusions	p. 42
A6	Selected client references	Four comparable healthcare finance programs	p. 44

A

SUPPORTING DETAIL

Appendix

Six reference items, A1 to A6, on pages 36 to 44.



A

SUPPORTING DETAIL

Appendix

Six reference items, A1 to A6, on pages 36 to 44.



APPENDIX

Reference material

Detail for readers who want to check the numbers and the sources behind the proposal.

A1 Detailed close calendar

Day-by-day view of the current close for 14 entities

A2 Entity readiness assessment

Scores for data, process and system readiness

A3 Control framework

Key controls mapped to the target operating model

A4 Tooling options

Comparison of three automation platforms

Self-scheduling splits into four features and nine stories

EPIC E-101

Self-scheduling

Patients book, move and cancel visits in the app without a call.

33

points, four features

31

acceptance criteria

	USER STORY	AC	POINTS	STATUS
FIND	As a patient, I want to search open slots by provider	4	3	Done
	As a patient, I want to filter by location and time	3	3	Done
BOOK	As a patient, I want to pick a visit type and book	5	8	In progress
	As a patient, I want a booking confirmation	3	2	In progress
CHANGE	As a patient, I want to reschedule from the app	4	5	In progress
	As a patient, I want to cancel and free the slot	3	3	Not started
NOTIFY	As a patient, I want a text reminder with a link	4	5	Not started
	As a patient, I want to add the visit to my calendar	2	2	Not started
	As a patient, I want a day-before prep note	3	2	Blocked

Source: Northfield digital front door backlog, Oct 2026. Sample figures for layout only.

Eight epics carry the front door; access and estimates lead

	EPIC	OUTCOME	PRIORITY	SIZE	STATUS	RELEASE
ACCESS	Self-scheduling	Book and move visits without a call	P1	XL	In progress	4.1 · Nov
	Waitlist and cancellations	Refill released slots within hours	P2	L	In progress	4.2 · Jan
	Digital intake	Arrive with forms and consents done	P1	L	In progress	4.1 · Nov
MONEY	Cost estimates	Show the patient price before the visit	P1	XL	Not started	4.2 · Jan
	Bill pay	Pay a statement in two taps	P2	M	Done	4.0 · Oct
	Payment plans	Set up a plan without calling billing	P3	L	Not started	4.3 · Mar
ENGAGE	Secure messaging	Reach the care team in one thread	P2	L	Blocked	4.2 · Jan
	Proxy access	Let caregivers act for family members	P4	M	Not started	4.3 · Mar

Source: Northfield digital front door backlog, Oct 2026. Sample figures for layout only.

The next two sprints are ready; the tail is aging

42

ready items, of 118 open

88%

refined for the next two sprints

47

days average backlog age

■ AT RISK

6

blocked items

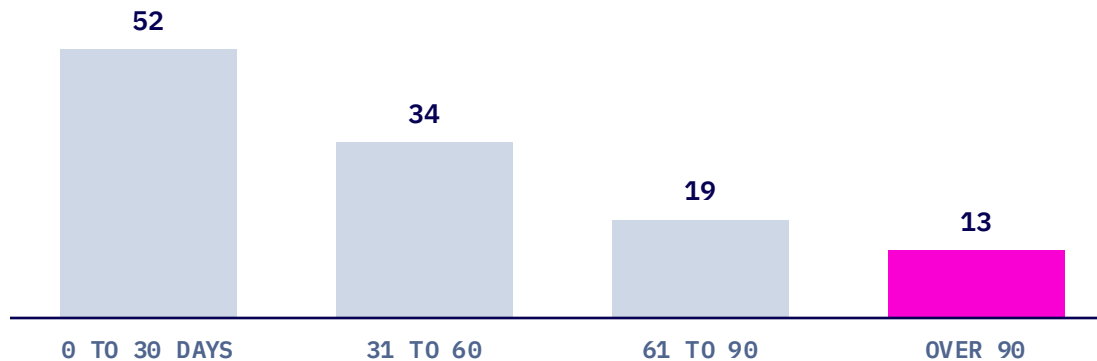
■ AT RISK

28%

of open items are bugs,
72% features

Open items by age

ITEMS



01

Refinement is on track

Next two sprints hold 88% refined stories.

02

Messaging is stuck

Five of six blocked items wait on the vendor API.

03

Old items need a verdict

13 items over 90 days: close, split or schedule.

Source: Northfield digital front door backlog, Oct 2026. Sample figures for layout only.

A story must be ready to start and done to count

DEFINITION OF READY

A story enters a sprint when

- ✓ Story states who, what and why
- ✓ Acceptance criteria are testable
- ✓ Designs are linked and reviewed
- ✓ Dependencies are named and clear
- ✓ Estimated by the team at 8 points or less
- ✓ Product owner has ranked it
- ✓ Fits in one sprint

DEFINITION OF DONE

A story is done when

- ✓ Code reviewed and merged
- ✓ Unit and UI tests pass
- ✓ Accessibility check passed
- ✓ Analytics events verified
- ✓ Release notes drafted
- ✓ Product owner accepted it
- ✓ Support team briefed

The product owner owns ready, the delivery team owns done, and both are reviewed in the quarterly retro.

Source: Northfield digital front door backlog, Oct 2026. Sample figures for layout only.

Three releases take the front door through March

RELEASE 4.1 • NOV 2026

01 Book and arrive ready

- Self-scheduling · 33 pts
- Digital intake · 20 pts

OUTCOME

Patients book, move and arrive with forms done.

RELEASE 4.2 • JAN 2027

02 Know the price first

- Cost estimates · 34 pts
- Waitlist offers · 18 pts
- Messaging · 8 pts

OUTCOME

Patients see the price before they commit.

RELEASE 4.3 • MAR 2027

03 Pay and share care

- Payment plans · 21 pts
- Proxy access · 13 pts
- Hardening · 14 pts

OUTCOME

Patients settle bills and share care with family.

53 of 60

points planned against capacity

60 of 62

points planned against capacity

48 of 60

points planned against capacity

Source: Northfield digital front door backlog, Oct 2026. Sample figures for layout only.

RANKED BACKLOG

Twelve ranked items; the top five fill two sprints

- **Value** patient time saved and calls avoided.
- **Urgency** payer and vendor dates that cannot move.
- **Effort** team sizing, in S to XL tags.
- **Risk** unblock the messaging vendor first.

CUT LINE

Top 5

fit 48 points, two sprints at capacity.

#	BACKLOG ITEM	EPIC	PRIORITY	SIZE	SPRINT
1	Saved cards for bill pay	Bill pay	P1	S	S25
2	Reschedule from reminder	Self-scheduling	P1	M	S25
3	Open-slot filters	Self-scheduling	P2	M	S26
4	Estimate at booking	Cost estimates	P2	XL	S26
5	Intake pre-fill	Digital intake	P2	L	S26
6	Waitlist offers	Waitlist	P2	L	S27
7	Message threads	Messaging	P2	L	S27
8	Payment plan setup	Payment plans	P3	L	S27
9	Proxy invite reminders	Proxy access	P3	S	Later
10	Message attachments	Messaging	P3	M	Later
11	Family calendar view	Proxy access	P4	M	Later
12	Wayfinding maps	Wayfinding	P4	XL	Later

Sprints 24 to 27 commit to 24 points each

Sprint 24

OCT 12 TO 23 • CAPACITY 28

Saved cards · 4
Reschedule link · 6
Cancel, free slot · 4
Open-slot filters · 6
Confirmation · 4

Sprint 25

OCT 26 TO NOV 6 • CAPACITY 28

Estimate API · 8
Intake pre-fill · 8
Calendar add · 2
Waitlist offers · 6

Sprint 26

NOV 9 TO 20 • CAPACITY 28

Estimate screen · 8
Message threads · 6
Prep note · 4
Bug burn-down · 6

Sprint 27

NOV 30 TO DEC 11 • CAPACITY 28

Payment plan setup · 8
Proxy invites · 4
Attachments · 4
Hardening · 6

COMMITMENT 24 PTS

 Carryover from Sprint 23

 Planned story, points shown after the name

Reschedule from a text reminder in under a minute

USER STORY

As a patient, I want to move a visit from the reminder text, so that I keep my care without calling.

ACCEPTANCE CRITERIA

GIVEN	WHEN	THEN
A reminder text 48 hours out	I tap the link and sign in	My visit opens with a Reschedule action
Open slots with the same provider	I choose a new slot	The old slot is freed and I get a confirmation
No open slots in 14 days	I tap Reschedule	I am offered the waitlist or a call-back

Designs: Figma link placeholder, reschedule flow

Source: Northfield digital front door backlog, Oct 2026. Sample figures for layout only.

5 pts

estimate, planned for Sprint 25

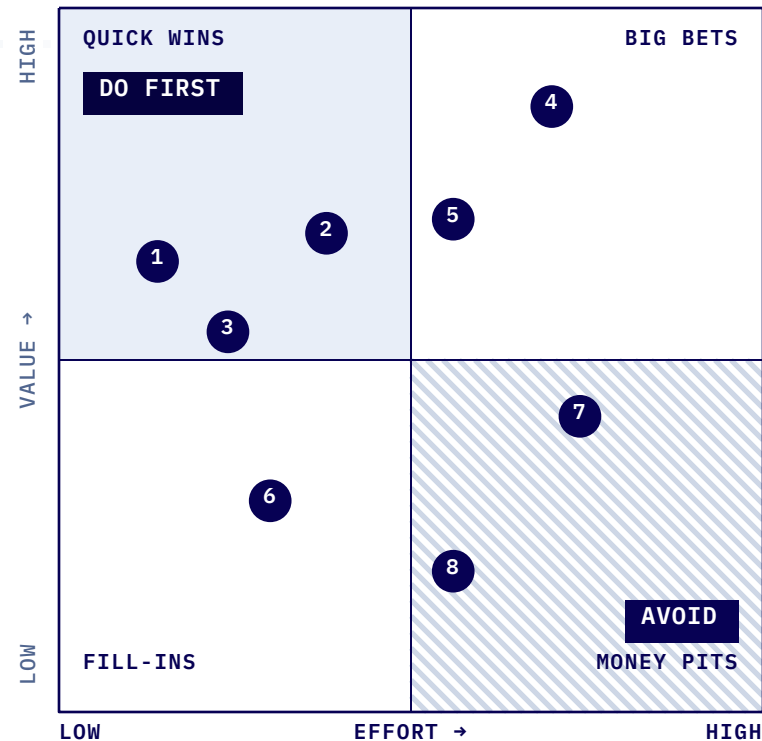
DONE	DEFINITION OF DONE
☒	Code reviewed and merged
☒	Unit and UI tests pass
☐	Accessibility check passed
☐	Product owner accepted

DEPENDS ON

- NFHS-198 slot search API
- Text reminder deep links

Three quick wins come first; wayfinding waits

Value and effort



RANKED BY VALUE OVER EFFORT

READING THE 2X2

1. Reschedule from reminder
2. Open-slot filters
3. Saved cards for bill pay
4. Estimate at booking
5. Intake pre-fill
6. Proxy invite reminders
7. Family calendar view
8. Wayfinding maps

- 01 Quick wins**
Items 1 to 3 go into the next sprint.
- 02 Big bets**
Items 4 and 5 are sliced before commitment.
- 03 Hold**
Items 6 to 8 wait for capacity.

Source: Northfield digital front door backlog, Oct 2026. Sample figures for layout only.

Weighted scoring puts bill pay and reschedule at the top

BACKLOG ITEM	VALUE	TIME CRITICALITY	RISK REDUCTION	JOB SIZE	WSJF SCORE	RANK	TIER
Saved cards for bill pay	5	5	3	2	6.5	1	P1
Reschedule from reminder	8	8	3	3	6.3	2	P1
Open-slot filters	5	3	2	3	3.3	3	P2
Estimate at booking	13	13	8	13	2.6	4	P2
Intake pre-fill	8	5	5	8	2.2	5	P2
Proxy invite reminders	3	2	3	5	1.6	6	P3
Message attachments	3	2	2	5	1.4	7	P3
Wayfinding maps	2	1	1	8	0.5	8	P4

WSJF = (value + time criticality + risk reduction) divided by job size, each on a 1, 2, 3, 5, 8, 13 scale.

Source: Northfield digital front door backlog, Oct 2026. Sample figures for layout only.



THANK YOU • NORTHFIELD HEALTH SYSTEM

Clear Action. Lasting Impact.

westmonroe.com



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NEXT STEPS

Three steps take us from decision to a January kick-off

01 Confirm scope

Agree wave 1 entities and the executive sponsor by 31 October.

02 Mobilize the team

Name your process owners and set the kick-off agenda in November.

03 Kick off wave 1

Start discovery with the five highest-delay entities in January.

DECISION REQUESTED

Approve the wave 1 scope and name an executive sponsor by 31 October.

YOUR CONTACTS

Engagement partner

name@westmonroe.com

Delivery director

name@westmonroe.com

Finance transformation lead

name@westmonroe.com



NEXT STEP

Kick off wave 1 in January

Engagement partner
name@westmonroe.com

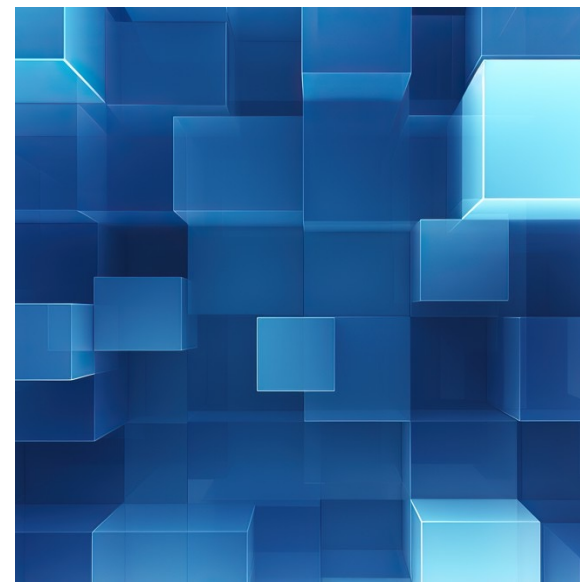
Questions?

Ask now, or reach us at any time. We will answer within one working day.

PROPOSAL CONTACTS

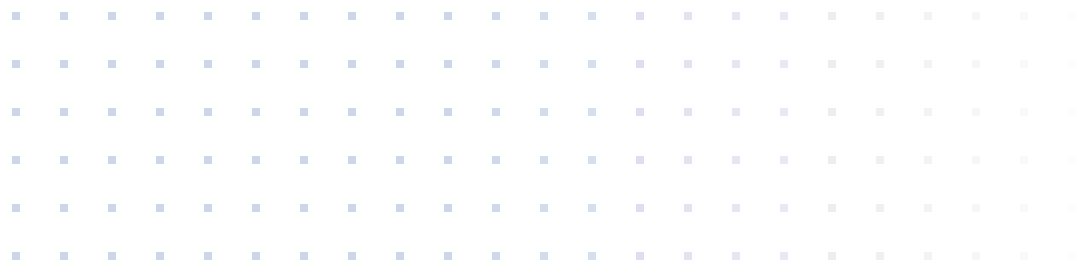
Engagement partner
name@westmonroe.com

Delivery director
name@westmonroe.com



Thank you

We would welcome the chance to start in January.



Engagement partner

name@westmonroe.com



Delivery director

name@westmonroe.com



Finance transformation lead

name@westmonroe.com

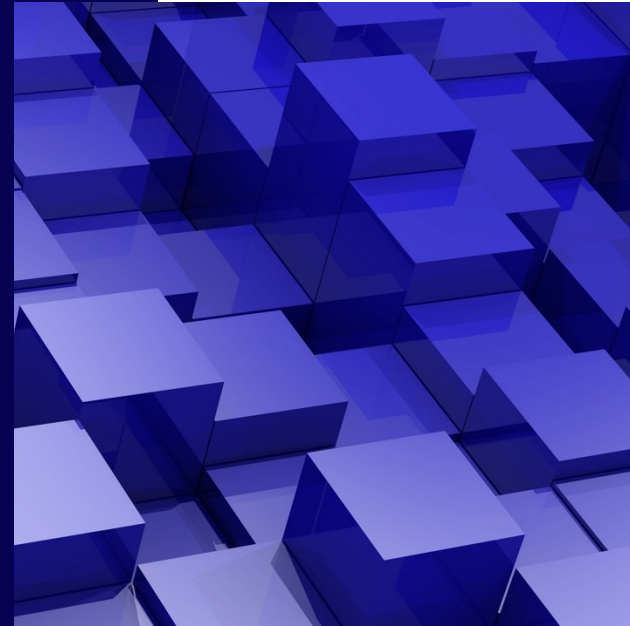


PROPOSAL FOR NORTHFIELD HEALTH SYSTEM

Close the books in five days

PREPARED BY

West Monroe · Finance Transformation



PROPOSAL CONTACTS

Engagement partner
name@westmonroe.com

Delivery director
name@westmonroe.com





PROPOSAL FOR NORTHFIELD HEALTH SYSTEM

Close the books in five days

PREPARED BY

West Monroe · Finance Transformation

PROPOSAL CONTACTS

Engagement partner
name@westmonroe.com

Delivery director
name@westmonroe.com

DATE

October 2026





RESPONSE TO RFP NHS-2026-014

Close the books in five days

PREPARED BY

West Monroe · Finance Transformation

PROPOSAL CONTACTS

Engagement partner
name@westmonroe.com

Delivery director
name@westmonroe.com

RESPONSE TO

RFP NHS-2026-014

ISSUED BY

Northfield Finance Office

SUBMITTED

October 2026

VALID THROUGH

31 January 2027





PROPOSAL • OCTOBER 2026

Close the books in **five days**

PREPARED BY

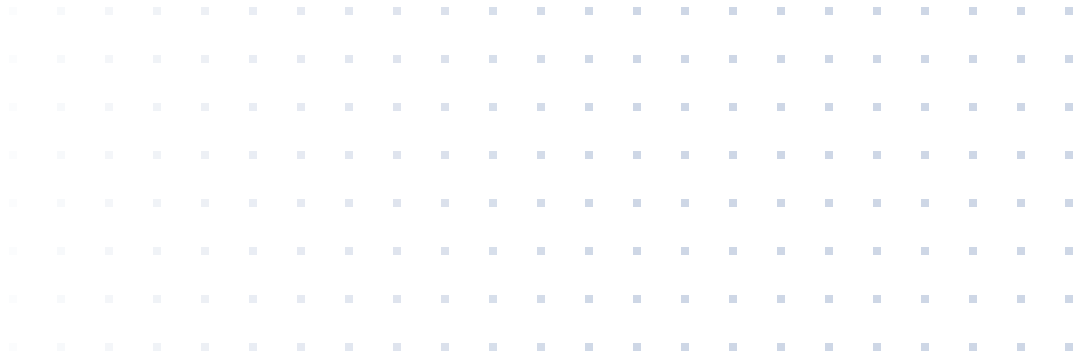
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Finance Transformation

PROPOSAL CONTACTS

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PROPOSAL FOR NORTHFIELD HEALTH SYSTEM

The path to a five-day close

PREPARED BY

West Monroe
Finance Transformation

PROPOSAL CONTACTS

Engagement partner
name@westmonroe.com

Delivery director
name@westmonroe.com

DATE

October 2026





WORKSHOP • NORTHFIELD HEALTH SYSTEM

Close redesign working session



DATE

14 October 2026

LOCATION

Northfield main campus

WHO IS IN THE ROOM

Finance, shared services, IT

FACILITATED BY

Engagement partner • name@westmonroe.com

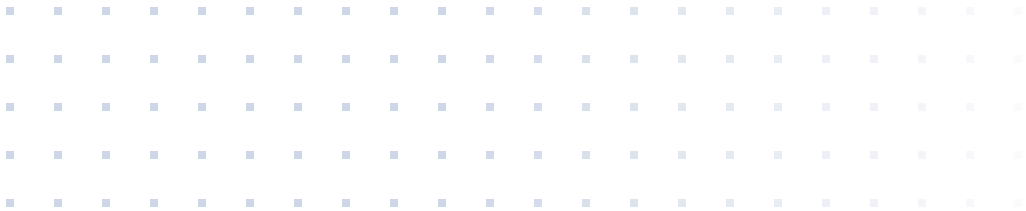


03

SECTION 03 OF 06

The solution

The operating model, tooling and controls that bring the close down to five days.



01 Our understanding

02 Approach

03 Solution

Target operating model
Systems and automation
Controls and audit readiness
Change and adoption

04 Team

05 Commercials

06 Why West Monroe

Approach: how the work gets delivered

NARRATIVE OF THIS SECTION

Approved outcomes become working capability in four subphases. Design fixes the target, build and validation prove it with Northfield teams, and cutover moves entities in controlled waves behind leader sign-off.

SLIDE	TITLE	WHAT IT COVERS	WHEN TO USE
31	Approach at a glance	Four subphases take approved outcomes to a live capability	Opening any delivery talk
32	Subphase detail	Activities, outputs and owners for each subphase	Buyer asks for specifics
33	Working model	What each Northfield team does, week by week	Resourcing questions
34	Phased cutover	Entities move in controlled waves, not one event	Risk-averse operations leaders
35	Decision gates	Where leaders sign off before work moves on	Steering committee review
36	Delivery risks	Top risks and how we contain each one	Late-stage diligence
37	Governance cadence	Who meets, how often and what they decide	Sponsor wants oversight

Approach: how the work gets delivered

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Approach: how the work gets delivered

SECTION 03 OF 05

Approved outcomes become working capability in four subphases.

Design fixes the target, build and validation prove it with Northfield teams, and cutover moves entities in controlled waves behind leader sign-off.

TIME TO PRESENT

Five slides, about 12 minutes.

WHAT'S INSIDE

- 31 Approach at a glance**
Four subphases take approved outcomes to a live capability.
- 32 Subphase detail**
Activities, outputs and owners for each subphase.
- 33 Working model**
What each Northfield team does, week by week.
- 34 Phased cutover**
Entities move in controlled waves, not one event.
- 35 Decision gates**
Where leaders sign off before work moves on.

Twelve appendix items, A1 to A12

REF	EXHIBIT	WHAT IT CONTAINS	PAGE
A1	Interview summary	Who we spoke with and the themes that emerged	65
A2	Close calendar detail	Task-level calendar for each entity, day 1 to day 12	66
A3	Entity-level findings	Findings and baselines for each of the eight entities	67
A4	Reconciliation inventory	All balance-sheet reconciliations with owners and frequency	68
A5	Process maps	Current-state and future-state maps for the close	69
A6	Systems inventory	Applications, interfaces and data owners in scope	70
A7	Control matrix	Key controls mapped to risks and audit assertions	71
A8	Detailed work plan	Activities, durations and dependencies by wave	72
A9	Fee build-up	How the phase fees are built from team and effort	73
A10	Staffing roster by role	Roles, allocation and timing across the three waves	74
A11	Glossary of terms	Definitions used throughout the proposal	75
A12	Case study detail	A comparable health system close transformation	76

Pages 63 and 64 hold the appendix divider and this index.

Twenty exhibits, figures and tables, by page

E01	Northfield entities and volumes	7	E11	Close calendar, day by day	32
E02	Goals and success measures	8	E12	Systems landscape	33
E03	Close cycle by entity, days	9	E13	Tooling options compared	35
E04	Where the days go in a close	11	E14	Recommended architecture	36
E05	Root causes ranked by impact	12	E15	Delivery team structure	46
E06	Three-wave approach	16	E16	Fees by phase	51
E07	Integrated timeline	20	E17	Business case	53
E08	Governance structure	21	E18	Payback and sensitivity	54
E09	Risks and mitigations	23	E19	Comparable client results	58
E10	Target operating model	27	E20	Client reference summary	60

Solution: eighteen slides in three parts

3.1	Operating model	27–32	3.2	Technology	33–38	3.3	Readiness	39–44
	Target operating model	27		Systems landscape	33		Roles after the redesign	39
	Process redesign overview	28		Automation opportunities	34		Controls and audit readiness	40
	Reconciliation redesign	29		Tooling options compared	35		Training and enablement	41
	Intercompany simplification	30		Recommended architecture	36		Cutover plan	42
	Journal entry workflow	31		Data and integration design	37		Hypercare and handover	43
	Close calendar: day by day	32		Reporting and dashboards	38		Benefits tracking	44

Before: Approach, pages 15–26

After: Team, pages 45–50

Seven sections, 76 pages: where each part sits

SECTION	WHAT IT COVERS	PAGES	SLIDES	SHARE OF DECK
00 Cover and indexes	Title page and the three-page slide index	1–4	4	5%
01 Our understanding	Your goals, the close today and what success looks like	5–14	10	13%
02 Approach	Three waves, timeline, governance and risks	15–26	12	16%
03 Solution	Operating model, systems, controls and cutover	27–44	18	24%
04 Team	Leadership, delivery team and your team's role	45–50	6	8%
05 Commercials	Fees, assumptions, business case and next steps	51–56	6	8%
06 Why West Monroe	Healthcare finance experience, results and references	57–62	6	8%
07 Appendix	Twelve supporting exhibits, A1 to A12	63–76	14	18%

76 pages in all. Solution is the largest section at 18 slides; the appendix holds the supporting detail.

Every slide in the proposal, by section and page

03	Solution (continued)	27–44	03	Solution (continued)	27–44
	Reconciliation redesign	29		Cutover plan	42
	Intercompany simplification	30		Hypercare and handover	43
	Journal entry workflow	31		Benefits tracking	44
	Close calendar: day by day	32	04	Team	45–50
	Systems landscape	33		Engagement leadership	45
	Automation opportunities	34		Delivery team structure	46
	Tooling options compared	35		Core team profiles	47
	Recommended architecture	36		Specialist advisors	48
	Data and integration design	37		Your team's role	49
	Reporting and dashboards	38		Staffing by phase	50
	Roles after the redesign	39	05	Commercials	51–56
	Controls and audit readiness	40		Fees by phase	51
	Training and enablement	41		Assumptions and dependencies	52

Every slide in the proposal, by section and page

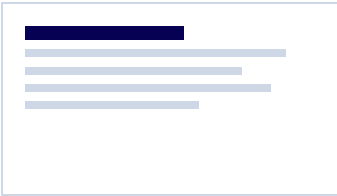
05	Commercials (continued)	51–56	07	Appendix (continued)	63–76
	Business case	53		Appendix index	64
	Payback and sensitivity	54		Interview summary	65
	Commercial terms	55		Close calendar detail	66
	Next steps and timeline	56		Entity-level findings	67
06	Why West Monroe	57–62		Reconciliation inventory	68
	Healthcare finance experience	57		Process maps	69
	Comparable client results	58		Systems inventory	70
	Our delivery model	59		Control matrix	71
	Client references	60		Detailed work plan	72
	Our commitments to you	61		Fee build-up	73
	Why West Monroe	62		Staffing roster by role	74
07	Appendix	63–76		Glossary of terms	75
	Appendix divider	63		Case study detail	76

Every slide in the proposal, by section and page

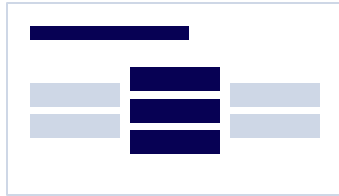
01	Our understanding	5–14	02	Approach (continued)	15–26
	Executive summary	5		Wave 1: stabilize the close	17
	What we heard from your leaders	6		Wave 2: standardize entities	18
	Northfield at a glance	7		Wave 3: automate and sustain	19
	Goals for the next 24 months	8		Integrated timeline	20
	The close cycle today	9		Governance and cadence	21
	Pain points by entity	10		Decision rights	22
	Where the days go	11		Risks and mitigations	23
	Root causes behind the delay	12		How we work with your team	24
	Constraints we will design around	13		Change and adoption plan	25
	What success looks like	14		Quality and controls approach	26
02	Approach	15–26	03	Solution	27–44
	Guiding principles	15		Target operating model	27
	Our approach in three waves	16		Process redesign overview	28

Team and commercials at a glance

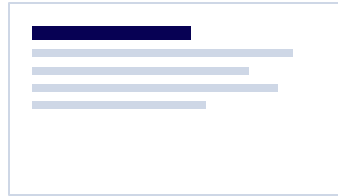
04 TEAM • PAGES 45–50



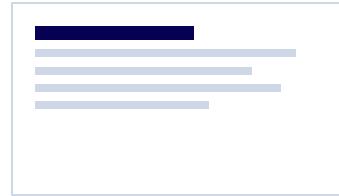
45 • Engagement leadership



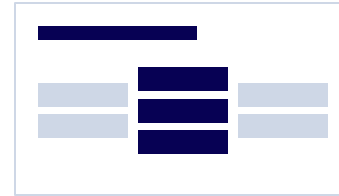
46 • Delivery team structure



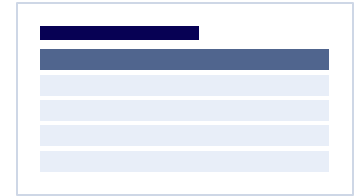
47 • Core team profiles



48 • Specialist advisors

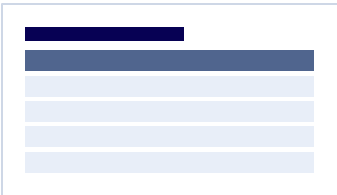


49 • Your team's role

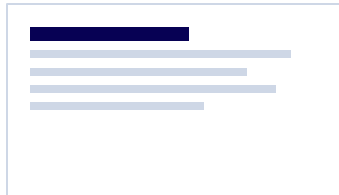


50 • Staffing by phase

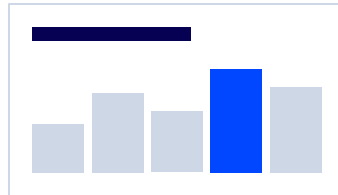
05 COMMERCIALS • PAGES 51–56



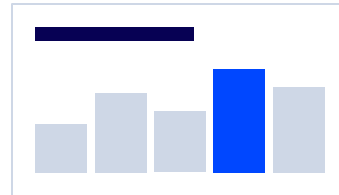
51 • Fees by phase



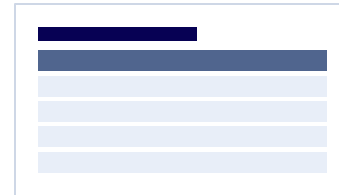
52 • Assumptions and dependencies



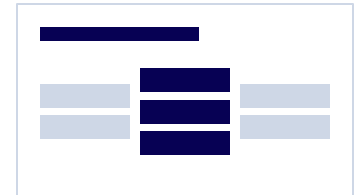
53 • Business case



54 • Payback and sensitivity



55 • Commercial terms



56 • Next steps and timeline

Every slide with its type and page

SLIDES 5-20 OF 76

	SLIDE	TITLE	TYPE	PAGE
UNDERSTANDING	1.1	Executive summary	SUMMARY	5
	1.2	What we heard from your leaders	INSIGHT	6
	1.3	Northfield at a glance	CONTEXT	7
	1.4	Goals for the next 24 months	GOALS	8
	1.5	The close cycle today	DIAGNOSTIC	9
	1.6	Pain points by entity	TABLE	10
	1.7	Where the days go	CHART	11
	1.8	Root causes behind the delay	DIAGNOSTIC	12
	1.9	Constraints we will design around	CONSTRAINTS	13
	1.10	What success looks like	OUTCOMES	14
APPROACH	2.1	Guiding principles	PRINCIPLES	15
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Every slide with its type and page

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CRITICAL PATH

Build and UAT carry no float; feeders have a week or two

FEEDER WORK • FLOAT

CRITICAL PATH • ZERO FLOAT

— Critical link — Feeds, with float

Data access granted
29 JAN • 1 WK FLOAT

Business case
19 FEB • 1 WK FLOAT

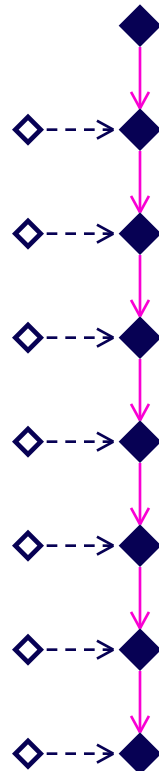
Interface specs signed
12 MAR • 2 WK FLOAT

Test scripts ready
26 MAR • 1 WK FLOAT

Training delivered
23 APR • 1 WK FLOAT

Wave 2 readiness
21 MAY • 1 WK FLOAT

Benefits checkpoint
18 JUN • 1 WK FLOAT



22 JAN
WEEK 2

Mobilize complete

Team, governance and plan in place; steering committee chartered.

5 FEB
WEEK 4

Current state signed off

Access and revenue cycle baseline agreed with operations leads.

26 FEB
WEEK 7

Design approved

Future-state scheduling, registration and billing design approved.

26 MAR
WEEK 11

Build complete

Workflows, rules and interfaces configured and unit tested.

16 APR
WEEK 14

UAT exit

Users pass every critical script with no open severity 1 or 2 defects.

30 APR
WEEK 16

Go-live wave 1

Central scheduling and two hospitals move to the new workflows.

28 MAY
WEEK 20

Go-live wave 2

The remaining hospitals and ambulatory clinics go live.

25 JUN
WEEK 24

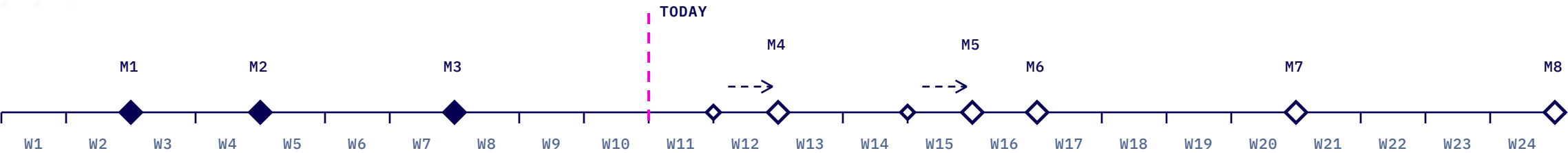
Hypercare exit

KPIs stable for four weeks; support handed to Northfield teams.

Two milestones slip a week, but the 30 Apr go-live holds

STATUS AT 19 MAR • WEEK 10 OF 24

◆ Done ■ On track ■ At risk — Baseline to forecast



MILESTONE		BASELINE	FORECAST	VARIANCE	STATUS
M1	Mobilize complete	22 Jan	22 Jan	On plan	Done
M2	Current state signed off	5 Feb	5 Feb	On plan	Done
M3	Design approved	26 Feb	26 Feb	On plan	Done
M4	Build complete	26 Mar	2 Apr	+1 wk	At risk
M5	UAT exit	16 Apr	23 Apr	+1 wk	At risk
M6	Go-live wave 1	30 Apr	30 Apr	On plan	On track
M7	Go-live wave 2	28 May	28 May	On plan	On track
M8	Hypercare exit	25 Jun	25 Jun	On plan	On track

VARIANCE TO BASELINE

Build and UAT slip one week

- Clearinghouse interface rework adds a week to build.
- Six-day UAT weeks recover it, so go-live holds at 30 Apr.

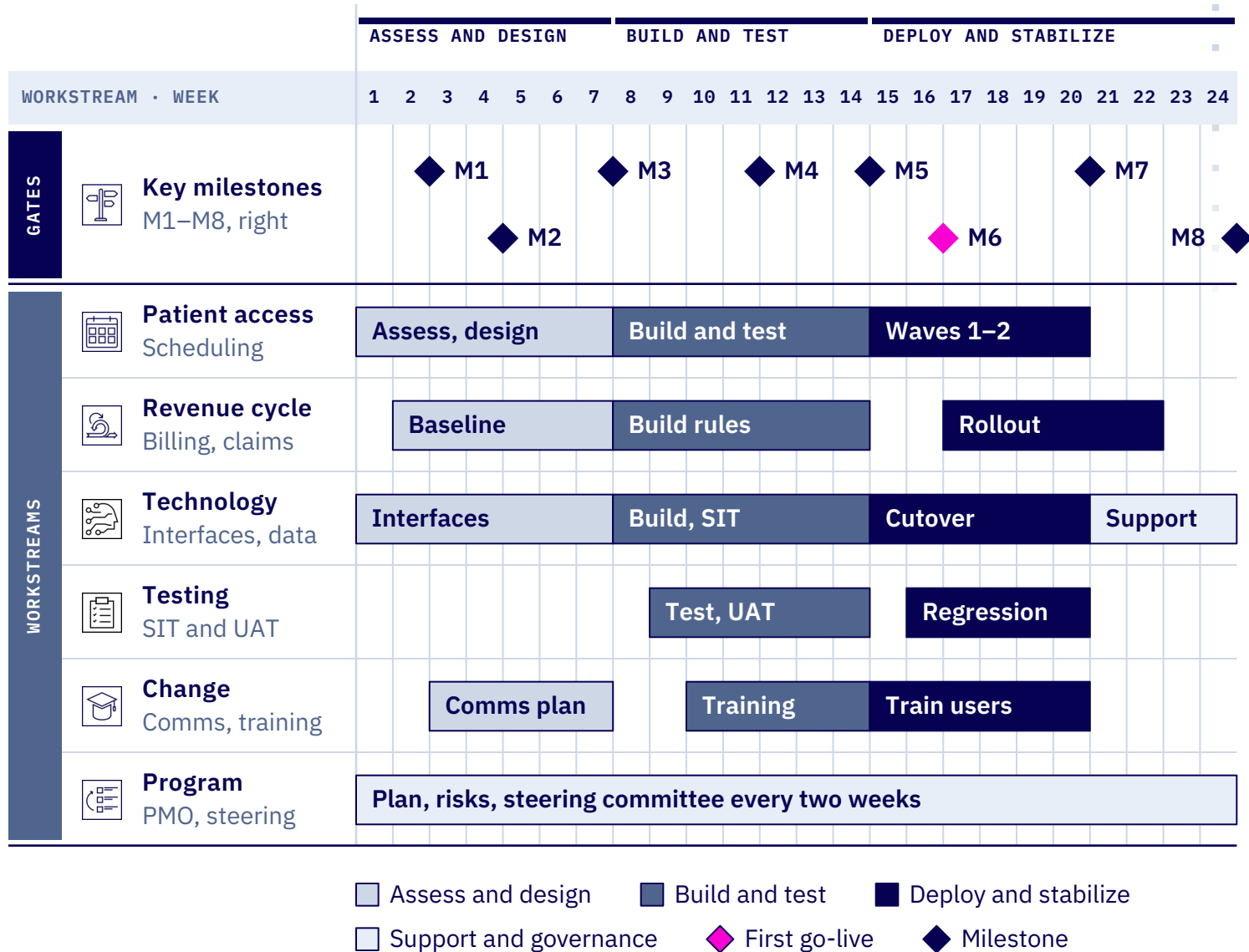
Four of 26 milestones are forecast a week late

	ID	MILESTONE	BASELINE	FORECAST	DESCRIPTION	OWNER	STATUS
PHASE 1	MS-01	M1 Mobilize complete	22 Jan	22 Jan	Team, governance and plan in place	PMO lead	Done
	MS-02	Data access granted	29 Jan	29 Jan	Read access to scheduling and billing data	IT applications lead	Done
	MS-03	Interviews complete	29 Jan	29 Jan	Leads interviewed at every site	VP patient access	Done
	MS-04	M2 Current state signed off	5 Feb	5 Feb	Access and revenue cycle baseline agreed	VP patient access	Done
	MS-05	Baseline KPIs agreed	12 Feb	12 Feb	No-show, denial and cash KPIs baselined	VP revenue cycle	Done
	MS-06	Future-state design drafted	19 Feb	19 Feb	Scheduling, registration and billing designs	Design lead	Done
	MS-07	Business case approved	26 Feb	26 Feb	Benefits and run cost approved by the CFO	CFO	Done
	MS-08	M3 Design approved	26 Feb	26 Feb	Steering committee signs the design	Steering committee	Done
PHASE 2	MS-09	Build specifications signed	5 Mar	5 Mar	Configuration specs for every workflow	IT applications lead	Done
	MS-10	Configuration demo	12 Mar	12 Mar	First workflows shown to operations leads	Product owner	Done
	MS-11	Interface specs signed	12 Mar	19 Mar	Clearinghouse and EHR specs agreed	Integration lead	Done
	MS-12	Test plan approved	19 Mar	19 Mar	SIT and UAT scope and entry criteria	Test lead	Done
	MS-13	Test scripts ready	26 Mar	26 Mar	Critical UAT scripts written and reviewed	Test lead	On track
	MS-14	M4 Build complete	26 Mar	2 Apr	Workflows, rules and interfaces unit tested	IT applications lead	At risk

Deployment milestones hold to the baseline dates

MILESTONE REGISTER, CONTINUED • 2 OF 2

	ID	MILESTONE	BASELINE	FORECAST	DESCRIPTION	OWNER	STATUS
PHASE 2	MS-15	Data conversion trial 2	2 Apr	9 Apr	Second full load reconciled to source totals	Data lead	 At risk
	MS-16	SIT exit	9 Apr	9 Apr	End-to-end flows pass across all interfaces	Test lead	 On track
	MS-17	M5 UAT exit	16 Apr	23 Apr	No open severity 1 or 2 defects	Test lead	 At risk
PHASE 3	MS-18	Training delivered	23 Apr	23 Apr	Wave 1 staff trained in the new workflows	Change lead	 On track
	MS-19	Cutover rehearsal complete	23 Apr	23 Apr	Dress rehearsal inside the cutover window	Cutover lead	 On track
	MS-20	Go/no-go decision	27 Apr	27 Apr	Steering committee confirms readiness	Steering committee	 On track
	MS-21	M6 Go-live wave 1	30 Apr	30 Apr	Central scheduling and two hospitals go live	Program director	 On track
	MS-22	Wave 1 stabilized	14 May	14 May	Queues and work lists back to baseline	Hypercare lead	 On track
	MS-23	Wave 2 readiness confirmed	21 May	21 May	Sites, users and data ready for wave 2	Change lead	 On track
	MS-24	M7 Go-live wave 2	28 May	28 May	Remaining hospitals and clinics go live	Program director	 On track
	MS-25	Benefits checkpoint	18 Jun	18 Jun	KPIs measured against the business case	VP revenue cycle	 On track
	MS-26	M8 Hypercare exit	25 Jun	25 Jun	Support handed to Northfield teams	VP revenue cycle	 On track

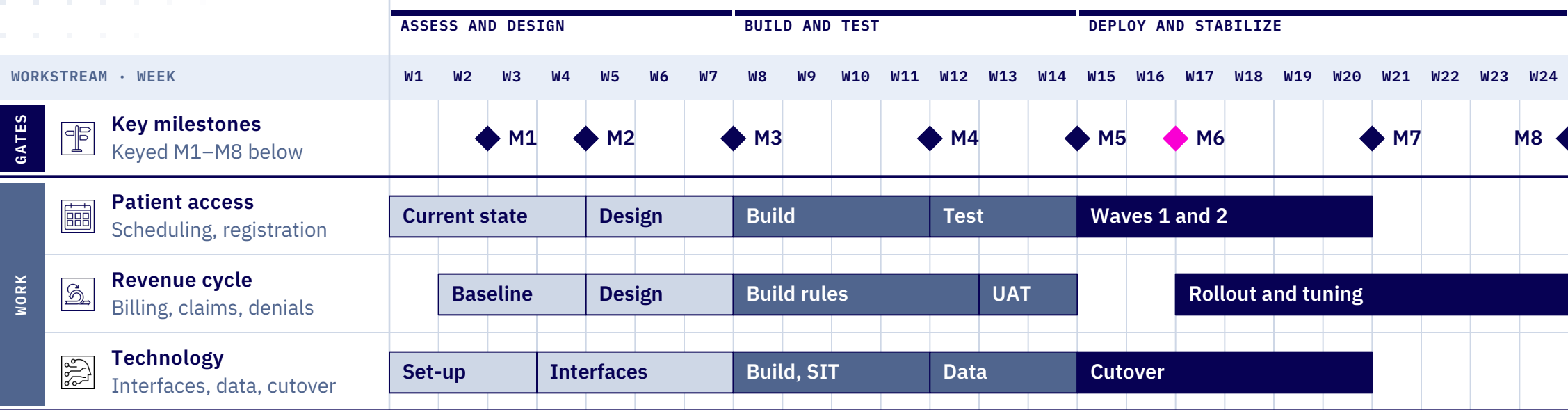


WORK PLAN

Eight gates, two go-live waves

- M1 Mobilize complete**
22 Jan · Team and plan in place
- M2 Current state signed off**
5 Feb · Baseline agreed
- M3 Design approved**
26 Feb · Future state approved
- M4 Build complete**
26 Mar · Configured and unit tested
- M5 UAT exit**
16 Apr · No open severity 1–2 defects
- M6 Go-live wave 1**
30 Apr · Central scheduling, two hospitals
- M7 Go-live wave 2**
28 May · All hospitals and clinics
- M8 Hypercare exit**
25 Jun · KPIs stable, support handed over

Eight gates hold the plan to a 24-week finish



□ Assess and design ■ Build and test ■ Deploy and stabilize ◆ First go-live ◆ Milestone

M1	Mobilize complete	22 JAN	Team and plan in place	M5	UAT exit	16 APR	Critical scripts passed
M2	Current state signed off	5 FEB	Baseline agreed	M6	Go-live wave 1	30 APR	Two hospitals live
M3	Design approved	26 FEB	Future state approved	M7	Go-live wave 2	28 MAY	All sites live
M4	Build complete	26 MAR	Configured and tested	M8	Hypercare exit	25 JUN	Support handed over

PROGRAM TIMELINE

Eight milestones take the program live in 24 weeks

PHASE 1 • ASSESS AND DESIGN

W1-W7 • 11 JAN – 26 FEB

M1 • 22 JAN

Mobilize complete

Team, governance and plan in place; steering committee chartered.

PHASE 2 • BUILD AND TEST

W8-W14 • 1 MAR – 16 APR

M3 • 26 FEB

Design approved

Future-state scheduling, registration and billing design approved.

PHASE 3 • DEPLOY AND STABILIZE

W15-W24 • 19 APR – 25 JUN

M5 • 16 APR

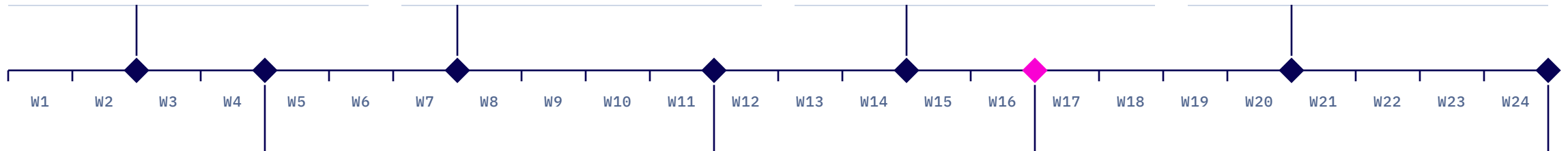
UAT exit

Users pass every critical script with no open severity 1 or 2 defects.

M7 • 28 MAY

Go-live wave 2

The remaining hospitals and ambulatory clinics go live.



M2 • 5 FEB

Current state signed off

Access and revenue cycle baseline agreed with operations leads.

M4 • 26 MAR

Build complete

Workflows, rules and interfaces configured and unit tested.

M6 • 30 APR

Go-live wave 1

Central scheduling and two hospitals move to the new workflows.

M8 • 25 JUN

Hypercare exit

KPIs stable for four weeks; support handed to Northfield teams.

PHASE 2 • BUILD AND TEST

Build and test closes on two milestones in seven weeks

PHASE 2 OF 3 • WEEKS 8-14 • 1 MAR – 16 APR

Configure, integrate and prove the new workflows. The phase ends when business users pass UAT.



M4 • 26 MAR • WEEK 11

Build complete

Workflows, rules and interfaces configured and unit tested.

EXIT CRITERIA

- All stories accepted
- Interfaces pass unit tests

M5 • 16 APR • WEEK 14

UAT exit

Business users pass every critical script.

EXIT CRITERIA

- No open severity 1 or 2 defects
- Cutover runbook rehearsed

CHECKPOINTS IN THIS PHASE

- 5 MAR Build specs signed
Configuration specs for every workflow
- 12 MAR Interface specs signed
Clearinghouse and EHR specs agreed
- 19 MAR Test plan approved
SIT and UAT scope and entry criteria
- 9 APR SIT exit
End-to-end flows pass on all interfaces

Every phase ends on milestones the steering group signs

PHASE 1 • W1-W7 • 11 JAN – 26 FEB

Assess and design

M1 • 22 JAN • WEEK 2

◆ Mobilize complete

Team, governance and plan in place.

M2 • 5 FEB • WEEK 4

◆ Current state signed off

Baseline agreed with operations leads.

M3 • 26 FEB • WEEK 7

◆ Design approved

Steering committee approves the future state.

EXIT CRITERIA

- Current state and design signed off
- Business case approved by the CFO

PHASE 2 • W8-W14 • 1 MAR – 16 APR

Build and test

C1 • 12 MAR • WEEK 9

◇ Configuration demo

First workflows shown to operations leads.

M4 • 26 MAR • WEEK 11

◆ Build complete

Workflows, rules and interfaces unit tested.

M5 • 16 APR • WEEK 14

◆ UAT exit

No open severity 1 or 2 defects remain.

EXIT CRITERIA

- Every critical UAT script passed
- Cutover runbook rehearsed end to end

PHASE 3 • W15-W24 • 19 APR – 25 JUN

Deploy and stabilize

M6 • 30 APR • WEEK 16

◆ Go-live wave 1

Central scheduling and two hospitals go live.

M7 • 28 MAY • WEEK 20

◆ Go-live wave 2

Remaining hospitals and clinics go live.

M8 • 25 JUN • WEEK 24

◆ Hypercare exit

Support handed to Northfield teams.

EXIT CRITERIA

- Both waves live at every site
- KPIs stable for four weeks

Two milestones land in February and two in April

Q1 • W1-W3

January

◆ M1 • 22 JAN

Mobilize complete

Team, governance and plan in place; steering committee chartered.

PMO LEAD

DELIVERABLES

- Mobilization plan
- Governance charter

Q1 • W4-W7

February

◆ M2 • 5 FEB

Current state signed off

Baseline agreed with operations leads.

VP PATIENT ACCESS

◆ M3 • 26 FEB

Design approved

Steering committee approves the future state.

STEERING COMMITTEE

Q1 • W8-W12

March

◆ M4 • 26 MAR

Build complete

Workflows, rules and interfaces configured and unit tested.

IT APPLICATIONS LEAD

DELIVERABLES

- Build specifications
- Configured workflows

Q2 • W13-W16

April

◆ M5 • 16 APR

UAT exit

No open severity 1 or 2 defects remain.

TEST LEAD

◆ M6 • 30 APR

Go-live wave 1

Central scheduling and two hospitals go live.

PROGRAM DIRECTOR

Q2 • W17-W20

May

◆ M7 • 28 MAY

Go-live wave 2

The remaining hospitals and ambulatory clinics go live.

PROGRAM DIRECTOR

DELIVERABLES

- Wave 2 readiness

Q2 • W21-W24

June

◆ M8 • 25 JUN

Hypercare exit

KPIs stable for four weeks; support handed to Northfield teams.

VP REVENUE CYCLE

DELIVERABLES

- Hypercare report

PROGRAM TIMELINE

Each milestone closes with the deliverables that prove it

22 JAN WEEK 2	◆	Mobilize complete Team, governance and plan in place; steering committee chartered.	MOBILIZATION PLAN GOVERNANCE CHARTER
5 FEB WEEK 4	◆	Current state signed off Access and revenue cycle baseline agreed with operations leads.	CURRENT-STATE REPORT BASELINE KPIS
26 FEB WEEK 7	◆	Design approved Future-state scheduling, registration and billing design approved.	FUTURE-STATE DESIGN BUSINESS CASE
26 MAR WEEK 11	◆	Build complete Workflows, rules and interfaces configured and unit tested.	BUILD SPECIFICATIONS CONFIGURED WORKFLOWS
16 APR WEEK 14	◆	UAT exit Users pass every critical script with no open severity 1 or 2 defects.	TEST PLAN UAT SIGN-OFF
30 APR WEEK 16	◆	Go-live wave 1 Central scheduling and two hospitals move to the new workflows.	TRAINING MATERIALS CUTOVER RUNBOOK
28 MAY WEEK 20	◆	Go-live wave 2 The remaining hospitals and ambulatory clinics go live.	WAVE 2 READINESS
25 JUN WEEK 24	◆	Hypercare exit KPIs stable for four weeks; support handed to Northfield teams.	HYPERCARE REPORT

PROGRAM TIMELINE

Two go-live waves in 24 weeks, each gated

A 24-week program moves scheduling, registration and billing at Northfield onto one set of workflows, live in two waves.

W1-W7 · 11 JAN - 26 FEB

Assess and design

Baseline, future-state design and business case.

W8-W14 · 1 MAR - 16 APR

Build and test

Configure, integrate and prove it in UAT.

W15-W24 · 19 APR - 25 JUN

Deploy and stabilize

Two go-live waves, then four weeks of hypercare.

22 JAN
WEEK 2

5 FEB
WEEK 4

26 FEB
WEEK 7

26 MAR
WEEK 11

16 APR
WEEK 14

30 APR
WEEK 16

28 MAY
WEEK 20

25 JUN
WEEK 24



Mobilize complete

Team, governance and plan in place.

MOBILIZATION PLAN

GOVERNANCE CHARTER



Current state signed off

Baseline agreed with operations leads.

CURRENT-STATE REPORT

BASELINE KPIS



Design approved

Steering committee approves the future state.

FUTURE-STATE DESIGN

BUSINESS CASE



Build complete

Workflows, rules and interfaces unit tested.

BUILD SPECIFICATIONS

CONFIGURED WORKFLOWS



UAT exit

No open severity 1 or 2 defects remain.

TEST PLAN

UAT SIGN-OFF



Go-live wave 1

Central scheduling and two hospitals go live.

TRAINING MATERIALS

CUTOVER RUNBOOK



Go-live wave 2

Remaining hospitals and clinics go live.

WAVE 2 READINESS



Hypercare exit

Support handed to Northfield teams.

HYPERCARE REPORT

Four deliverables are accepted and three are in review

DELIVERABLE	DUE	REVIEWER	APPROVER	ACCEPTANCE CRITERIA	STATUS
D01 Program charter and plan	W2 · Nov 13	PMO lead	Steering committee	Scope, plan, RACI signed by sponsor	■ Accepted
D02 Stakeholder and change map	W2 · Nov 13	Access director	PMO lead	All 6 sites and 14 roles mapped	■ Accepted
D03 Current-state assessment	W6 · Dec 11	Revenue cycle VP	Steering committee	Validated in 3 site walk-throughs	■ Accepted
D04 Baseline metrics pack	W6 · Dec 11	Finance director	CFO	12 months reconciled to the GL	■ Accepted
D05 Future-state design	W10 · Jan 8	Revenue cycle VP	Steering committee	Every design decision has an owner	■ In review
D06 Business case	W10 · Jan 8	Finance director	CFO	Benefits tied to the baseline pack	■ In review
D07 Build specifications	W15 · Feb 12	WM architect	CIO	Specs traced to approved design	■ In review
D08 Test plan and scripts	W15 · Feb 12	QA manager	CIO	Scripts cover all in-scope flows	□ Not started
D09 UAT exit report	W17 · Feb 26	QA manager	Steering committee	No open severity 1 or 2 defects	□ Not started
D10 Training materials	W18 · Mar 5	Access director	Revenue cycle VP	90% of wave 1 staff trained	□ Not started
D11 Cutover runbook	W18 · Mar 5	IT apps lead	CIO	Two dress rehearsals run to time	□ Not started
D12 Wave 2 readiness review	W21 · Mar 26	PMO lead	Steering committee	All wave 2 sites scored ready	□ Not started
D13 Hypercare report	W24 · Apr 16	PMO lead	Steering committee	Metrics stable for 4 weeks	□ Not started

Six milestones carry the program to a stable go-live

M2 • W6 • DEC 11

Current state signed off

Assessment findings and the baseline are accepted.

- **Assessment:** 6 areas
- **Baseline:** denials, AR, cash

M3 • W10 • JAN 8

Design approved

Target processes and the business case are approved.

- **Design:** processes, roles
- **Business case:** payback

M4 • W15 • FEB 12

Build complete

Configuration, interfaces and reports are built and unit tested.

- **Build specs:** traced to design
- **Test pack:** 320 scripts

M5 • W17 • FEB 26

UAT exit

Users accept the build and the go decision is recorded.

- **UAT exit report:** no Sev 1 or 2
- **Defect log:** owners, dates

M6 • W18 • MAR 5

Go-live wave 1

Three sites go live with trained staff and a command center.

- **Training:** role guides
- **Runbook:** rollback plan

M8 • W24 • APR 16

Hypercare exit

Metrics hold at baseline and support takes over.

- **Hypercare report:** 4 weeks stable
- **Handover:** to support

MILESTONES AND DELIVERABLES

Four sign-offs gate the program, each backed by named deliverables

The steering committee meets the week after each sign-off. Deliverables reach members five working days before.

PHASE 1 • W1-W6

Assess

2 milestones • 4 deliverables

PHASE 2 • W7-W17

Design and build

3 milestones • 5 deliverables

PHASE 3 • W18-W24

Deploy and stabilize

3 milestones • 4 deliverables

M2 • W6 • DEC 11

Current state signed off

Findings and baseline accepted.

- **Current-state assessment:** 6 areas
- **Baseline metrics pack:** 12 months
- **Change map:** 14 roles
- **Site walk-throughs:** 3 sites

M3 • W10 • JAN 8

Design approved

Target design and business case approved.

- **Future-state design:** processes, roles
- **Business case:** payback by wave
- **Decision log:** 42 decisions
- **Workshop outputs:** 4 sessions

M5 • W17 • FEB 26

UAT exit

Users accept the build; go decision recorded.

- **UAT exit report:** no Sev 1 or 2
- **Defect log:** owners, dates
- **Build specifications:** as built
- **Cutover runbook:** rehearsed

M8 • W24 • APR 16

Hypercare exit

Metrics stable; support takes over.

- **Hypercare report:** 4 weeks stable
- **Wave 2 readiness:** lessons applied
- **Support handover:** signed
- **Lessons learned:** both waves

Milestones and deliverables: mobilize to build complete

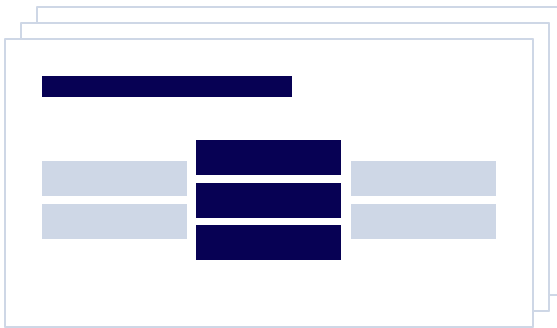
DELIVERABLE	DESCRIPTION	FORMAT	OWNER	APPROVER
M1 • MOBILIZE COMPLETE • W2 • NOV 13				
D01 Program charter and plan	Scope, governance and 24-week plan	PLAN	WM program lead	Steering committee
D02 Stakeholder and change map	Roles, impacts and cadence by site	WORKBOOK	WM change lead	PMO lead
M2 • CURRENT STATE SIGNED OFF • W6 • DEC 11				
D03 Current-state assessment	Process, systems and data baseline	REPORT	WM process lead	Steering committee
D04 Baseline metrics pack	Denials, errors, cash and days in AR	DASHBOARD	WM analytics lead	CFO
M3 • DESIGN APPROVED • W10 • JAN 8				
D05 Future-state design	Target processes, roles and systems	DESIGN DOC	WM design lead	Steering committee
D06 Business case	Benefits, cost and payback by wave	MODEL	WM program lead	CFO
M4 • BUILD COMPLETE • W15 • FEB 12				
D07 Build specifications	Configuration, interface, report specs	SPECS	IT apps lead	CIO
D08 Test plan and scripts	Entry and exit criteria, 320 scripts	TEST PACK	WM test lead	CIO

Milestones and deliverables: UAT exit to hypercare exit

MILESTONES AND DELIVERABLES • CONTINUED

DELIVERABLE	DESCRIPTION	FORMAT	OWNER	APPROVER
M5 • UAT EXIT • W17 • FEB 26				
D08 Test plan and scripts, final	Rerun results and the defect log	TEST PACK	WM test lead	CIO
D09 UAT exit report	Results, defects, go or no-go view	REPORT	WM test lead	Steering committee
M6 • GO-LIVE WAVE 1 • W18 • MAR 5				
D10 Training materials	Role guides, e-learning, super-users	COURSEWARE	Training lead	Revenue cycle VP
D11 Cutover runbook	Cutover, rollback, command center	RUNBOOK	WM cutover lead	CIO
M7 • GO-LIVE WAVE 2 • W21 • MAR 26				
D12 Wave 2 readiness review	Site readiness, wave 1 lessons	SCORECARD	WM change lead	Steering committee
D11 Cutover runbook, wave 2	Updated for wave 2 sites and lessons	RUNBOOK	WM cutover lead	CIO
M8 • HYPERCARE EXIT • W24 • APR 16				
D13 Hypercare report	Stability metrics, support handover	REPORT	WM program lead	Steering committee

Future-state design: the blueprint the build is held to



PURPOSE

Sets the target processes, roles and system changes the build is specified, tested and accepted against.

FORMAT

Design document, about 60 pages

AUDIENCE

Steering committee, area leads, IT

SUPPORTS M3 • W10 • JAN 8

Design approved

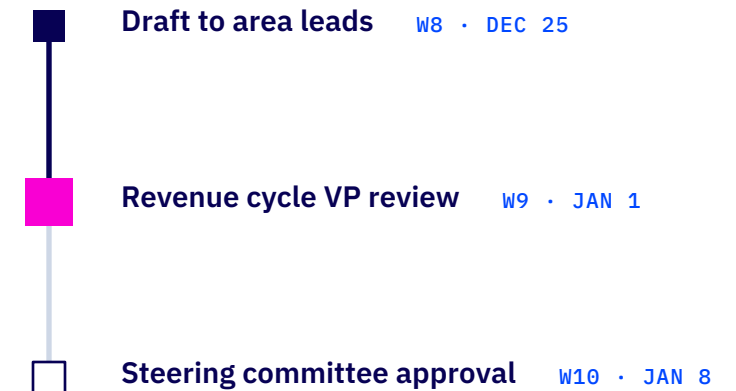
WHAT IT CONTAINS

- 01 Design principles**
Eight rules every decision is tested against
- 02 Future-state processes**
Scheduling to cash posting, 6 areas
- 03 Roles and staffing**
Who does each step, by site
- 04 System changes**
Configuration, interfaces and reports
- 05 Decision log**
42 decisions, each with an owner
- 06 Benefits link**
Each change tied to a baseline metric

INPUTS AND DEPENDENCIES

- **Current-state assessment** (D03), signed off W6
- **Baseline metrics pack** (D04)
- **Design workshops** four, weeks 7 to 9
- **Vendor roadmap** from IT apps lead

REVIEW AND ACCEPTANCE



Thirteen deliverables, each owned and reviewed

	DELIVERABLE	DESCRIPTION	FORMAT	DUE	OWNER	REVIEWER
ASSESS	D01 Program charter and plan	Scope, governance and 24-week plan	PLAN	M1 · W2	WM program lead	PMO lead
	D02 Stakeholder and change map	Roles, impacts and cadence by site	WORKBOOK	M1 · W2	WM change lead	Access director
	D03 Current-state assessment	Process, systems and data baseline	REPORT	M2 · W6	WM process lead	Revenue cycle VP
	D04 Baseline metrics pack	Denials, errors, cash and days in AR	DASHBOARD	M2 · W6	WM analytics lead	Finance director
DESIGN AND BUILD	D05 Future-state design	Target processes, roles and systems	DESIGN DOC	M3 · W10	WM design lead	Revenue cycle VP
	D06 Business case	Benefits, cost and payback by wave	MODEL	M3 · W10	WM program lead	Finance director
	D07 Build specifications	Configuration, interface, report specs	SPECS	M4 · W15	IT apps lead	WM architect
	D08 Test plan and scripts	Entry and exit criteria, 320 scripts	TEST PACK	M4 · W15	WM test lead	QA manager
	D09 UAT exit report	Results, defects, go or no-go view	REPORT	M5 · W17	WM test lead	QA manager
DEPLOY	D10 Training materials	Role guides, e-learning, super-users	COURSEWARE	M6 · W18	Training lead	Access director
	D11 Cutover runbook	Cutover, rollback, command center	RUNBOOK	M6 · W18	WM cutover lead	IT apps lead
	D12 Wave 2 readiness review	Site readiness, wave 1 lessons	SCORECARD	M7 · W21	WM change lead	PMO lead
	D13 Hypercare report	Stability metrics, support handover	REPORT	M8 · W24	WM program lead	PMO lead

Hypercare exit re-tests the baseline and business case

		ASSESS		DESIGN AND BUILD			DEPLOY		
DELIVERABLE	DESCRIPTION	M1	M2	M3	M4	M5	M6	M7	M8
D01 Program charter and plan	Scope, governance and 24-week plan	✓							
D02 Stakeholder and change map	Roles, impacts and cadence by site	✓	✓						
D03 Current-state assessment	Process, systems and data baseline		✓						
D04 Baseline metrics pack	Denials, errors, cash and days in AR		✓						✓
D05 Future-state design	Target processes, roles and systems			✓					
D06 Business case	Benefits, cost and payback by wave			✓					✓
D07 Build specifications	Configuration, interface, report specs				✓				
D08 Test plan and scripts	Entry and exit criteria, 320 scripts				✓	✓			
D09 UAT exit report	Results, defects, go or no-go view					✓			
D10 Training materials	Role guides, e-learning, super-users						✓	✓	
D11 Cutover runbook	Cutover, rollback, command center						✓	✓	
D12 Wave 2 readiness review	Site readiness, wave 1 lessons							✓	
D13 Hypercare report	Stability metrics, support handover								✓
M1 MOBILIZE COMPLETE · W2 M5 UAT EXIT · W17		M2 CURRENT STATE SIGNED OFF · W6 M6 GO-LIVE WAVE 1 · W18		M3 DESIGN APPROVED · W10 M7 GO-LIVE WAVE 2 · W21		M4 BUILD COMPLETE · W15 M8 HYPERCARE EXIT · W24			

Eight milestones over 24 weeks, each with its deliverables

M1 • W2 • NOV 13

Mobilize complete

- Charter and plan
- Change map

SIGN-OFF • SPONSOR

M2 • W6 • DEC 11

Baseline signed off

- Current-state assessment
- Baseline metrics

SIGN-OFF • STEERING

M3 • W10 • JAN 8

Design approved

- Future-state design
- Business case

SIGN-OFF • STEERING

M4 • W15 • FEB 12

Build complete

- Build specifications
- Test plan, 320 scripts

SIGN-OFF • CIO

M5 • W17 • FEB 26

UAT exit

- UAT exit report
- Go decision record

SIGN-OFF • STEERING

M6 • W18 • MAR 5

Go-live wave 1

- Training materials
- Cutover runbook

SIGN-OFF • GO-LIVE BOARD

M7 • W21 • MAR 26

Go-live wave 2

- Wave 2 readiness review
- Runbook update

SIGN-OFF • GO-LIVE BOARD

M8 • W24 • APR 16

Hypercare exit

- Hypercare report
- Support handover

SIGN-OFF • STEERING

Each phase closes on milestones its deliverables prove

PHASE 1 • W1-W6

Assess

MILESTONES

- Nov 13** M1 Mobilize complete
- Dec 11** M2 Current state signed off

DELIVERABLES

- **Charter and plan:** scope, RACI, plan
- **Change map:** 14 roles, 6 sites
- **Current-state assessment:** 6 areas
- **Baseline metrics:** 12 months, reconciled

Exit: findings and baseline accepted

PHASE 2 • W7-W17

Design and build

MILESTONES

- Jan 8** M3 Design approved
- Feb 12** M4 Build complete
- Feb 26** M5 UAT exit

DELIVERABLES

- **Future-state design:** processes, roles
- **Business case:** payback by wave
- **Build specifications:** traced to design
- **Test plan:** 320 scripts
- **UAT exit report:** go or no-go

Exit: go decision recorded at UAT

PHASE 3 • W18-W24

Deploy and stabilize

MILESTONES

- Mar 5** M6 Go-live wave 1
- Mar 26** M7 Go-live wave 2
- Apr 16** M8 Hypercare exit

DELIVERABLES

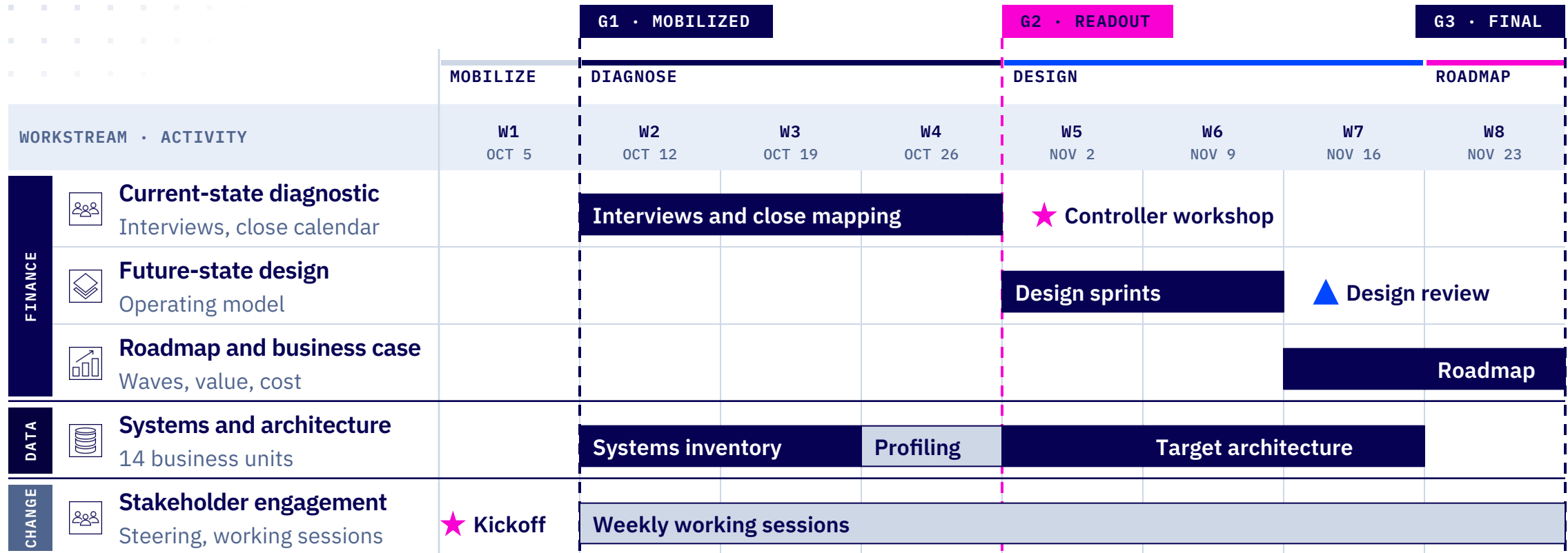
- **Training materials:** role guides, e-learning
- **Cutover runbook:** rollback, command center
- **Wave 2 readiness:** 6 sites scored
- **Hypercare report:** handover to support

Exit: support signs the handover

Eight milestones, each proved by named deliverables

	MILESTONE	DATE	DELIVERABLES	OWNER	ACCEPTANCE CRITERIA
ASSESS	M1 Mobilize complete	W2 · Nov 13	- Charter and plan: scope, RACI, 24-week plan - Change map: roles and impacts by site	WM program lead	Sponsor signs charter; team and data access in place
	M2 Current state signed off	W6 · Dec 11	- Current-state assessment: 6 areas - Baseline metrics: denials, AR, cash	Revenue cycle VP	Steering committee accepts findings and baseline
DESIGN AND BUILD	M3 Design approved	W10 · Jan 8	- Future-state design: processes and roles - Business case: payback by wave	WM design lead	All design decisions logged; CFO accepts business case
	M4 Build complete	W15 · Feb 12	- Build specs: config, interfaces, reports - Test plan: 320 scripts	IT apps lead	All specs built and unit tested; test pack ready
	M5 UAT exit	W17 · Feb 26	UAT exit report: go or no-go recommendation	WM test lead	No open severity 1 or 2 defects; go decision recorded
DEPLOY	M6 Go-live wave 1	W18 · Mar 5	- Training: role guides, super-user kit - Cutover runbook: rollback, command center	WM cutover lead	Wave 1 sites live; 90% of staff trained
	M7 Go-live wave 2	W21 · Mar 26	Wave 2 readiness review: 6 sites scored	WM change lead	Wave 2 sites live; wave 1 issues closed
	M8 Hypercare exit	W24 · Apr 16	Hypercare report: handover to support	PMO lead	Metrics at baseline or better for four weeks

Eight weeks from kickoff to a signed-off roadmap



■ Core activity □ Supporting activity ▨ Hatched: in progress, soft start or tentative ★ Workshop ▲ Review — Gate

Two roles are short at peak and one is over-resourced

ROLE	PEAK DEMAND (FTE)	PLANNED CAPACITY (FTE)	VARIANCE	STATUS
Engagement lead	0.5	0.5	—	■ Covered
Product manager	1	1	—	■ Covered
Solution architect	1	1	—	■ Covered
Data engineer	1	0.5	▼ -0.5	■ Gap
Business analysts	2	2	—	■ Covered
Developers	3	2	▼ -1.0	■ Gap
QA lead	1	1	—	■ Covered
Change lead	1	1.5	▲ +0.5	■ Over

Net peak position: 1.5 FTE short on developers and data engineering, 0.5 FTE over on change.

RESOURCE PLAN

Each firm role has a
named client
counterpart

- Counterparts hold decision rights for their area.
- Hours are per week at peak and fall in launch.
- A gap in any counterpart slows the matching firm role.

62 hrs

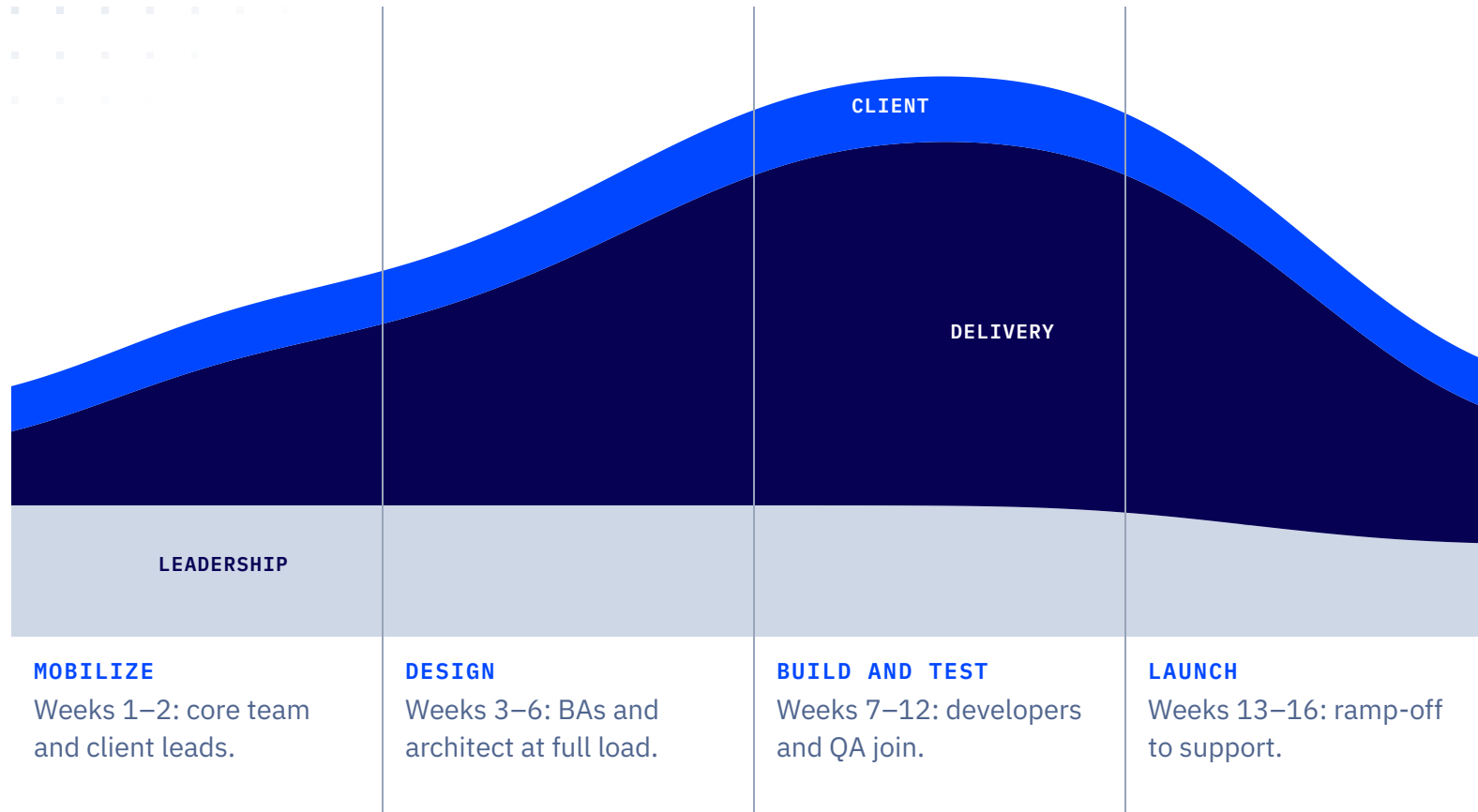
client time per week at peak

7

named counterparts

FIRM ROLE	CLIENT COUNTERPART	HRS PER WEEK
Engagement lead	Executive sponsor	2
Product manager	Product owner	20
Solution architect	IT integration lead	10
Data engineer	Data steward	6
Business analysts	Patient access SMEs	8
QA lead	UAT coordinator	10
Change lead	Department managers	6
Client commitment	Seven counterparts	62

The team peaks at 10.8 FTE, then rolls off



10.8 FTE

peak team size, weeks 7–10

7.9 FTE

average over 16 weeks

4.6 FTE

in week 16, handing over to run

1.2 FTE

client team at peak

A firm team of up to 9.5 FTE delivers in 16 weeks

ROLE (FIRM FTE)	MONTH 1	MONTH 2	MONTH 3	MONTH 4
Engagement lead	0.5	0.5	0.5	0.5
Product manager	1	1	1	0.5
Solution architect	1	1	0.5	0.25
Data engineer	0.5	1	1	0.5
Business analysts (2)	2	2	1	1
Developers (3)	–	3	3	1
QA lead	–	0.5	1	1
Change lead	0.5	0.5	1	1
Total firm FTE	5.5	9.5	9	5.75

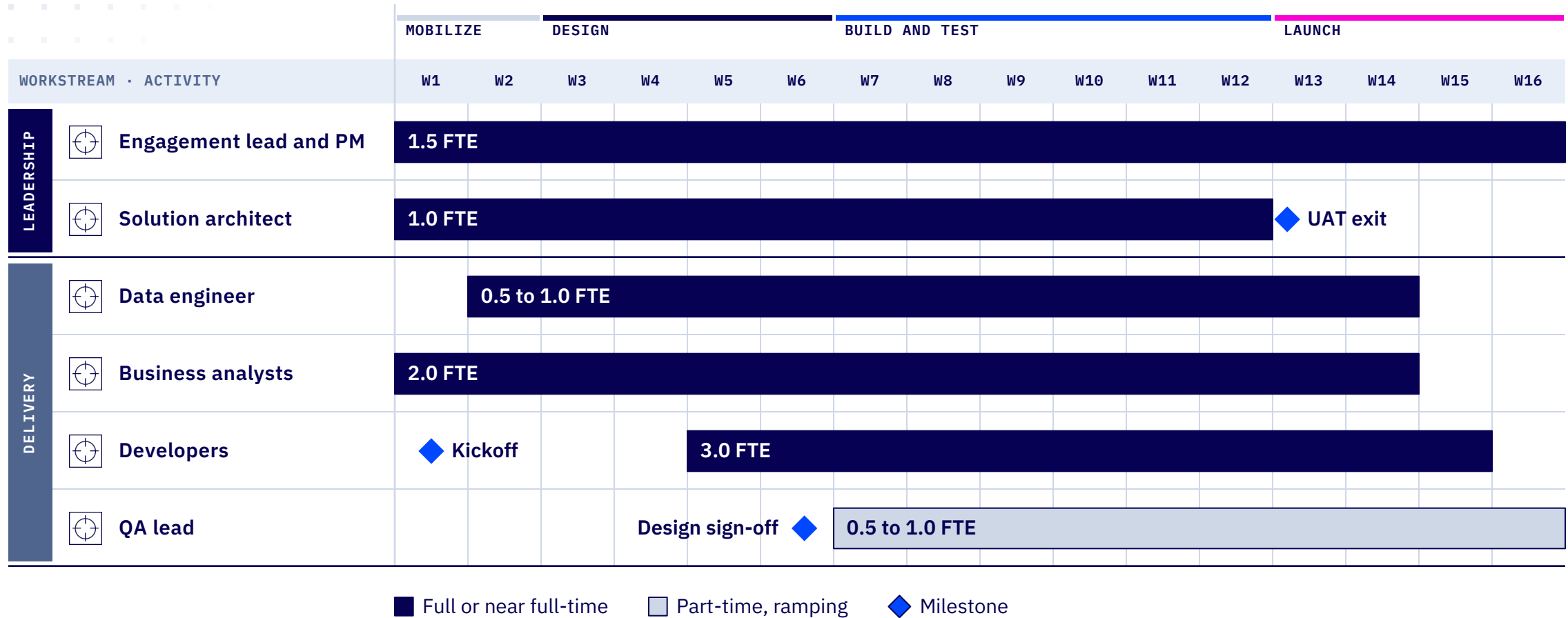
KEY ASSUMPTIONS

- Client leads are named by week 1.
- Developers start in week 5.
- Two-thirds of build is onshore.

PHASE	TEAM RUN RATE / MO
Mobilize	\$190K
Design	\$310K
Build and test	\$460K
Launch	\$330K
Peak monthly run rate	\$460K

Source: Sample figures for layout only; replace with current firm data before use.

Roles join as work needs them and roll off early



Nine named people are in place by February 16

MR **Mara R.**
Engagement lead

50% • JAN 5 – APR 24

DK **Devon K.**
Product manager

100% • JAN 5 – APR 17

SP **Sasha P.**
Solution architect

100% TO 25% • JAN 5 – MAR 27

TL **Tariq L.**
Data engineer

50% TO 100% • JAN 12 – APR 10

AN **Anya N.**
Business analyst

100% • JAN 5 – APR 10

JO **Jonas O.**
Business analyst

100% • JAN 5 – APR 10

PC **Priya C.**
Lead developer

100% • FEB 2 – APR 24

RB **Rowan B.**
QA lead

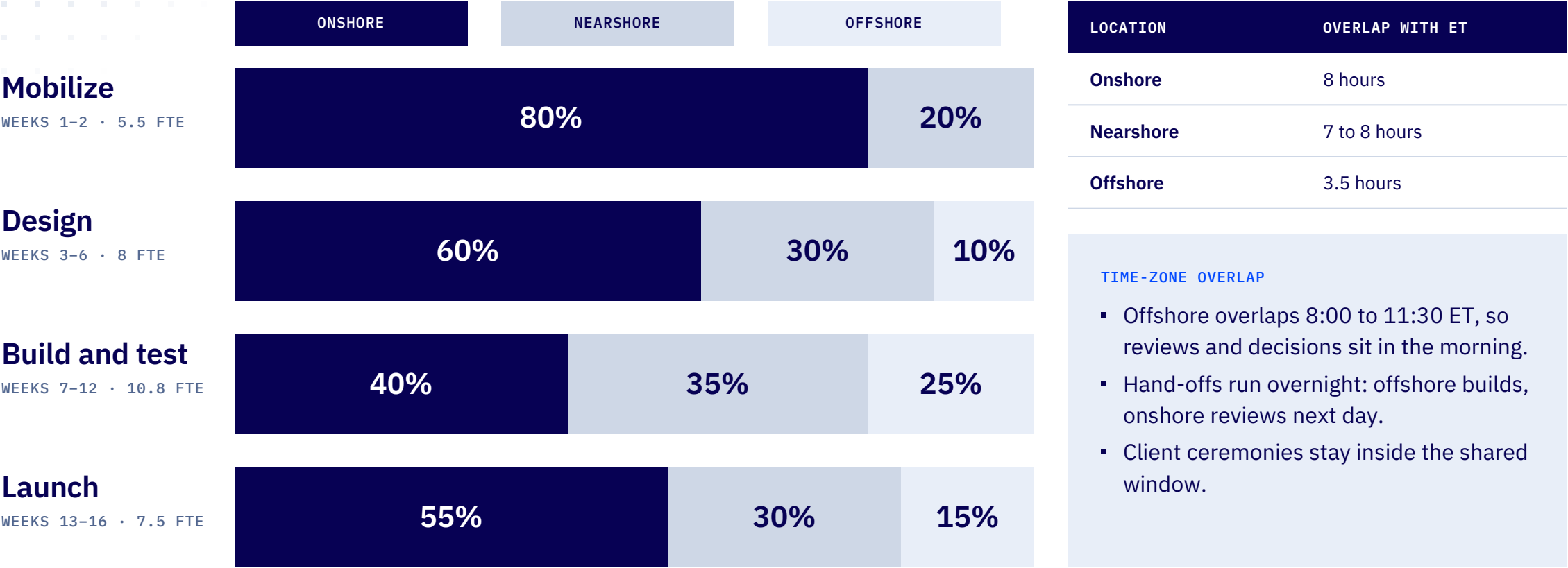
50% TO 100% • FEB 16 – APR 24

EH **Elena H.**
Change lead

50% TO 100% • JAN 19 – APR 24

Allocation is a share of a 40-hour week; ranges show ramp-up or ramp-down.

Offshore share grows as the build scales up



Source: Sample figures for layout only; replace with current firm data before use.

Loading peaks in weeks 7–8 and eases through launch

	ROLE	W1–2	W3–4	W5–6	W7–8	W9–10	W11–12	W13–14	W15–16	AVG FTE
LEADERSHIP	Engagement lead	0.5	0.5	0.5	0.5	0.5	0.5	0.5	0.5	0.5
	Product manager	1	1	1	1	1	1	1	0.5	0.94
	Solution architect	1	1	1	1	0.5	0.5	0.25	0.25	0.69
DELIVERY	Data engineer	0.5	0.5	1	1	1	1	0.5	0.5	0.75
	Business analysts (2)	2	2	2	2	1	1	1	1	1.5
	Developers (3)	–	–	2	3	3	3	2	1	1.75
	QA lead	–	–	0.5	0.5	1	1	1	1	0.62
	Change lead	0.5	0.5	0.5	0.5	1	1	1	1	0.75
CLIENT	Executive sponsor	0.1	0.1	0.1	0.1	0.1	0.1	0.1	0.1	0.1
	Product owner	0.5	0.5	0.5	0.5	0.5	0.5	0.5	0.5	0.5
	IT integration lead	0.25	0.25	0.5	0.5	0.5	0.5	0.25	0.25	0.38
	Total FTE	6.3	6.3	9.6	10.6	10.1	10.1	8.1	6.6	8.47

Peak fortnight is weeks 7–8 at 10.6 FTE; client counterparts hold near 1 FTE throughout.

Staffing peaks at 10.6 FTE in month 2

	ROLE	MONTH 1	MONTH 2	MONTH 3	MONTH 4	AVG FTE
LEADERSHIP	Engagement lead	0.5	0.5	0.5	0.5	0.5
	Product manager	1.0	1.0	1.0	0.5	0.9
	Solution architect	1.0	1.0	0.5	0.25	0.7
DELIVERY	Data engineer	0.5	1.0	1.0	0.5	0.8
	Business analysts (2)	2.0	2.0	1.0	1.0	1.5
	Developers (3)	–	3.0	3.0	1.0	1.8
	QA lead	–	0.5	1.0	1.0	0.6
	Change lead	0.5	0.5	1.0	1.0	0.8
CLIENT	Executive sponsor	0.1	0.1	0.1	0.1	0.1
	Product owner	0.5	0.5	0.5	0.5	0.5
	IT integration lead	0.25	0.5	0.5	0.25	0.4
	Total FTE	6.3	10.6	10.1	6.6	8.4

Peak is month 2: 10.6 FTE, of which the client team provides 1.1.

Every bet names its assumption and its decision date

HORIZON	STRATEGIC BET	ASSUMPTION TO VALIDATE	SIGNAL OF SUCCESS	DECISION POINT
NOW	Master patient index	Matching rules reach 6% duplicates without manual review at every site.	Duplicates under 8% at two hospitals by Q1 27.	Decide Mar 2027 Buy a matching service if above 9%.
NOW	Single sign-in	Patients and staff accept one identity without a second login.	70% of active patients enrolled within 90 days.	Decide Jun 2027 Fall back to staff-assisted enrolment.
NEXT	Self-scheduling	Patients book online when slots are real time and complete.	30% of primary care visits booked online.	Decide Sep 2027 Pivot to staff-assisted booking if under 20%.
NEXT	Digital intake	Patients finish forms and insurance before they arrive.	60% complete intake before the visit.	Decide Dec 2027 Cut to insurance capture only if under 40%.
LATER	Price estimates	Estimates within 10% of the final bill build trust.	Front-end denials under 6%.	Decide Jun 2028 Narrow to scheduled procedures if misses persist.

Source: Sample figures for layout only; replace with current firm data before use.

Three bets carry the vision into a sequenced roadmap

PRODUCT VISION

Every Northfield patient reaches, books and pays for care in one digital front door.

1 · Know every patient

OUTCOME

Duplicate records 11% → 6%

One record, one sign-in everywhere.

Now · Master patient index

Next · Single sign-in

Later · Shared patient data store

2 · Open the door

OUTCOME

Visits booked online 8% → 60%

Patients book and arrive ready, no call.

Now · Self-scheduling, primary care

Next · Digital intake

Later · Multilingual booking

3 · Pay and track with confidence

OUTCOME

Front-end denials 9% → 5%

Price and eligibility known up front.

Now · Eligibility checks

Next · Price estimates

Later · Referral tracking

Source: Sample figures for layout only; replace with current firm data before use.

PRODUCT STRATEGY ROADMAP

Every initiative traces back to the vision

PRODUCT VISION

Every Northfield patient reaches, books and pays for care in one digital front door.

NORTH STAR

80% of patients complete access online by 2028.

Baselines are 2026 averages across six hospitals.

Source: Sample figures for layout only; replace with current firm data before use.



	OUTCOME	BASELINE → TARGET	INITIATIVES	HORIZON
KNOW THE PATIENT	Duplicate patient records	11% → 6%	Master patient index	NOW
	Sites on one sign-in	0 → 6 of 6	Single sign-in	NOW
OPEN THE DOOR	Visits booked online	8% → 60%	Self-scheduling; digital intake	NEXT
	Calls to the access center	100% → 75%	Waitlist; reminders	NEXT
PAY AND TRACK	Front-end denials	9% → 5%	Eligibility checks; price estimates	LATER
	Referrals completed	61% → 85%	Referral tracking	LATER

Strategy to roadmap: pillars, bets and decision gates

STRATEGIC PILLAR

Now

Next

Later

OUTCOME • 11% → 6% DUPLICATES

Know every patient

Owner: chief data officer

DECISION • MAR 27

Master patient index

Signal: duplicates under 8%

DECISION • JUN 27

Single sign-in

Signal: 70% enrolled in 90 days

GATE • 2028

Shared data store

Waits for index and sign-in

OUTCOME • 8% → 60% BOOKED ONLINE

Open the door

Owner: patient access officer

DECISION • SEP 27

Self-scheduling

Signal: 30% of visits online

DECISION • DEC 27

Digital intake

Signal: 60% intake before visit

GATE • 2028

Multilingual booking

Booking in every language

OUTCOME • 9% → 5% DENIALS

Pay and track

Owner: VP revenue cycle

GATE • Q2 27

Eligibility checks

Signal: denials under 7%

DECISION • JUN 28

Price estimates

Signal: estimates within 10%

GATE • 2028

Referral tracking

One status view, order to visit

Source: Sample figures for layout only; replace with current firm data before use.

One north star, four inputs, the bets that move them

North star · patients who complete access online, 24% → 80% by 2028

Identity input

DUPLICATE RECORDS

11% → 6%

BETS THAT MOVE IT

NOW · Q4 26

Master patient index

NEXT · Q1 27

Single sign-in

Booking input

VISITS BOOKED ONLINE

8% → 60%

BETS THAT MOVE IT

NOW · Q1 27

Self-scheduling

LATER · 2028

Multilingual booking

Readiness input

INTAKE DONE BEFORE ARRIVAL

15% → 70%

BETS THAT MOVE IT

NEXT · Q2 27

Digital intake

NEXT · Q3 27

Smart reminders

Price clarity input

FRONT-END DENIALS

9% → 5%

BETS THAT MOVE IT

NEXT · Q2 27

Eligibility checks

LATER · Q3 27

Price estimates

Source: Sample figures for layout only; replace with current firm data before use.

Key results by quarter, each tied to its initiative

	OBJECTIVE	KEY RESULT	INITIATIVE MAPPED	OWNER	CONFIDENCE
Q4 26	Know every patient once	Duplicate records 11% → 9%	Master patient index	Chief data officer	High
		Sites on one sign-in 0 → 2 of 6	Single sign-in, pilot sites	Chief digital officer	Medium
Q1 27	Book without a call	Visits booked online 8% → 20%	Self-scheduling, primary care	Chief patient access officer	High
		Sites on one sign-in 2 → 4 of 6	Single sign-in, rollout	Chief digital officer	Medium
Q2 27	Arrive ready for the visit	Intake done before arrival 15% → 40%	Digital intake	Director of registration	Medium
		No-show rate 11% → 9%	Smart reminders	Chief patient access officer	Low

Source: Sample figures for layout only; replace with current firm data before use.

Every roadmap item is an outcome with a target

Now · Q4 26 to Q1 27

KPI · 11% → 6% DUPLICATE RECORDS

One record per patient

- Master patient index
- Match rules at six sites
- Duplicate review queue

KPI · 0 → 6 OF 6 SITES ON ONE SIGN-IN

One identity everywhere

- Single sign-in
- Staff identity federation
- Patient app login

Next · Q2 to Q3 27

KPI · 8% → 60% VISITS BOOKED ONLINE

Patients book without a call

- Self-scheduling, primary care
- Digital intake before arrival
- Waitlist and cancellations

KPI · 11% → 6% NO-SHOW RATE

Fewer visits lost to forgetting

- Smart reminders
- Care plans in the app
- Reminder channel choice

Later · Q4 27 and 2028

KPI · 9% → 5% FRONT-END DENIALS

Price known before booking

- Eligibility checks
- Price estimates
- Payment plans

KPI · 61% → 85% REFERRALS COMPLETED

Referrals followed to the visit

- Referral tracking
- Multilingual booking
- Caregiver proxy access

Source: Sample figures for layout only; replace with current firm data before use.

Strategy on a page: the digital front door to 2028

Every Northfield patient reaches, books and pays for care in one digital front door.

TARGET SEGMENTS

- Primary care patients booking routine visits
- Specialty patients arriving by referral
- Caregivers managing care for family
- Patients with chronic conditions

PROBLEMS WE SOLVE

- No booking without a phone call
- Six hospitals, six patient identities
- Prices unknown until the bill arrives
- Referrals lost between sites

VALUE PROPOSITION

FOR PATIENTS

One sign-in to find, book, prepare for and pay for care, with the price known first.

For staff: one verified record at every site.

STRATEGIC BETS

1. Know every patient once
2. Open the door online
3. Pay and track with confidence
4. Run on one data foundation

WHAT WE WILL NOT DO

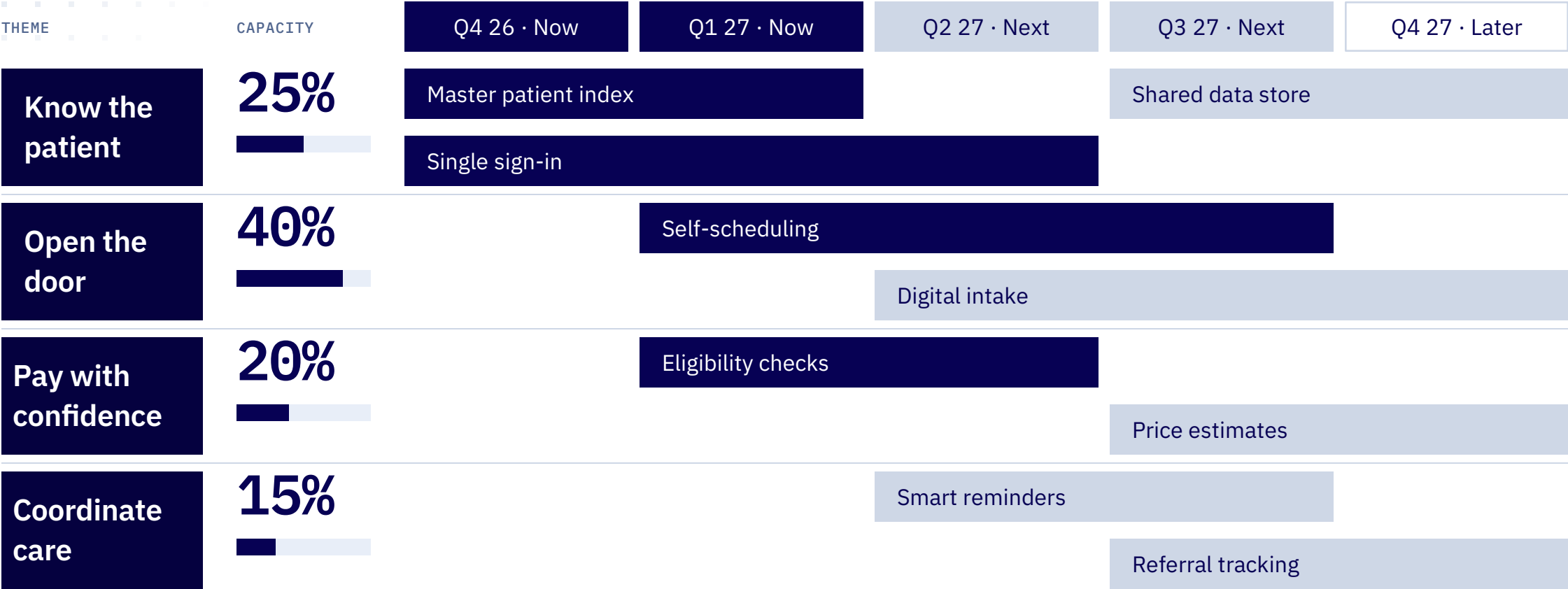
- Replace the EHR
- Build tools for a single hospital
- Add voice or kiosk channels before 2028
- Price every service before 2027

SUCCESS MEASURES

- 80% complete access online by 2028
- Calls to the access center down 25%
- Front-end denials 9% → 5%
- No-show rate 11% → 6%

Source: Sample figures for layout only; replace with current firm data before use.

Four themes share capacity across five quarters



Source: Sample figures for layout only; replace with current firm data before use.

Six sections and the detail behind each

01 Our understanding

Your goals and constraints
The close cycle today
Pain points by entity
What success looks like

03 Solution

Target operating model
Systems and automation
Controls and audit readiness
Change and adoption

05 Commercials

Fees by phase
Assumptions
Business case and payback

02 Approach

Guiding principles
Three-wave plan
Governance and cadence

04 Team

Engagement leadership
Delivery team
Your team's role

06 Why West Monroe

Healthcare finance experience
Comparable client results
References
Our commitment

Where to find each part of the proposal

01 Our understanding	3	04 Team	22
Your goals and constraints	3	Engagement leadership	22
The close cycle today	4	Delivery team	23
Pain points by entity	6	Your team's role	25
02 Approach	9	05 Commercials	26
Guiding principles	9	Fees by phase	26
Three-wave plan	10	Assumptions	27
Governance and cadence	12	Business case and payback	28
03 Solution	14	06 Why West Monroe	30
Target operating model	14	Healthcare finance experience	30
Systems and automation	15	Comparable client results	31
Controls and audit readiness	18	References	33

CONTENTS

What sits behind each section

Six sections follow. Appendix items A1 to A6 carry the supporting detail.



01 Our understanding

Your goals and constraints · The close cycle today · Pain points by entity · What success looks like

02 Approach

Guiding principles · Three-wave plan · Governance and cadence

03 Solution

Target operating model · Systems and automation · Controls and audit readiness · Change and adoption

04 Team

Engagement leadership · Delivery team · Your team's role

05 Commercials

Fees by phase · Assumptions · Business case and payback

06 Why West Monroe

Healthcare finance experience · Comparable client results · References · Our commitment

What each part of the proposal gives you

01 Our understanding

What we heard from Northfield leaders about the close, and where time and effort are lost today.

02 Approach

A three-wave plan that fixes the process first, then the systems.

03 Solution

The operating model, tooling and controls that bring the close down to five days.

04 Team

The leaders and specialists who will deliver, and how they work beside your finance team.

05 Commercial

Fees by phase, the assumptions behind them, and a business case that pays back inside 14 months.

06 Why West Monroe

Healthcare finance experience and results from comparable close transformations.

CONTENTS

What this proposal covers

Six sections, then supporting detail in the appendix.



01 Our understanding

What we heard from Northfield leaders about the close, and where time and effort are lost today.

02 Approach

A three-wave plan that fixes the process first, then the systems.

03 Solution

The operating model, tooling and controls that bring the close down to five days.

04 Team

The leaders and specialists who will deliver, and how they work beside your finance team.

05 Commercial

Fees by phase, the assumptions behind them, and a business case that pays back inside 14 months.

06 Why West Monroe

Healthcare finance experience and results from comparable close transformations.

Six sections, from understanding to proof

01	Our understanding	What we heard from Northfield leaders about the close, and where time and effort are lost today.
02	Approach	A three-wave plan that fixes the process first, then the systems.
03	Solution	The operating model, tooling and controls that bring the close down to five days.
04	Team	The leaders and specialists who will deliver, and how they work beside your finance team.
05	Commercials	Fees by phase, the assumptions behind them, and a business case that pays back inside 14 months.
06	Why West Monroe	Healthcare finance experience and results from comparable close transformations.